

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Double-Blind, Placebo-Controlled Study of Atezolizumab Plus Carboplatin and Etoposide With or Without Tiragolumab (Anti-Tigit Antibody) in Patients With Untreated Extensive-Stage Small Cell Lung Cancer **Short Title/Acronym:** SKYSCRAPER-02 **Protocol Number:** G041767 **NCT Number:** NCT04256421 **Trial Status:** Completed **Sponsor/Company:** Hoffmann-La Roche / Roche **Investigational Product:** Tiragolumab (MTIG7192A) + Atezolizumab + Carboplatin + Etoposide **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To determine whether the antitumor effect and survival benefits of the existing standard of care (atezolizumab plus carboplatin and etoposide [CE]) could be enhanced by the addition of the anti-TIGIT antibody tiragolumab in patients with untreated extensive-stage small cell lung cancer (ES-SCLC), measured by Progression-Free Survival (PFS) and Overall Survival (OS) in patients without a history/presence of brain metastases at baseline. **Secondary Objectives:** To evaluate PFS and OS in all randomly assigned patients (Full Analysis Set), Investigator-Assessed confirmed Objective Response Rate (ORR), Duration of Response (DOR), PFS rate at 6 and 12 months, OS rate at 12 and 24 months, and overall safety/tolerability.

2.2 Background & Rationale Current first-line treatment options for ES-SCLC are atezolizumab in combination with CE. Although these regimens have improved outcomes, most patients inevitably experience disease progression, and median survival remains at approximately 12 months. There is a significant unmet medical need for new therapeutic options to improve long-term outcomes in ES-SCLC. This study evaluates whether targeting the TIGIT immune checkpoint pathway alongside the PD-L1 pathway can overcome immune evasion and improve survival.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1, Stratified (by ECOG Performance Status [0 vs. 1], LDH levels [$\leq$ ULN vs. $>$ ULN], and presence/history of brain metastasis [yes vs. no]).

3.2 Planned Enrollment & Duration Target \$N\$: 490 participants
Number of Sites: 121 sites across 23 countries **Estimated Study Start:** 04-Feb-2020 **Estimated Primary Completion:** 06-Sep-2022

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years. 2. Histologically or cytologically confirmed Extensive-Stage Small Cell Lung Cancer (ES-SCLC). 3. No prior systemic treatment for ES-SCLC. 4. Measurable disease according to RECIST version 1.1 and an Eastern Cooperative Oncology Group (ECOG) performance status (PS) of 0 or 1. 5. Adequate hematologic and end-organ function. 6. Treatment-free for at least 6 months since last chemo/radiotherapy, among those treated (with curative intent) with prior chemo/radiotherapy for limited-stage SCLC.

4.2 Exclusion Criteria 1. Symptomatic or actively progressing central nervous system (CNS) metastases. 2. Malignancies other than small cell lung cancer (SCLC) within 5 years prior to randomization, with the exception of those with a negligible risk of metastasis or death treated with expected curative outcome. 3. Active or history of severe autoimmune disease or immune deficiency. 4. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis on screening chest computed tomography (CT) scan. 5. Positive test result for human immunodeficiency virus (HIV), or active hepatitis B or hepatitis C. 6. Severe infection at the time of randomization. 7. Treatment with any other investigational agent within 28 days prior to initiation of study treatment. 8. Prior treatment with CD137 agonists or immune checkpoint blockade therapies, anti-CTLA-4, anti-TIGIT, anti-PD-1, and anti-PD-L1 therapeutic antibodies. 9. Treatment with systemic immunostimulatory agents within 4 weeks or 5 drug elimination half-lives prior to randomization.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * *Induction:* Tiragolumab 600 mg IV (or Placebo) + Atezolizumab 1200 mg IV on Day 1 of each 21-day cycle. Carboplatin is administered followed by etoposide (trade names include Etopophos, Vepesid, Toposar) on Day 1, and etoposide is continued on Days 2 and 3 for 4 cycles. * *Maintenance:* Tiragolumab + Atezolizumab (or Placebo + Atezolizumab) administered on Day 1 of each 21-day cycle. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** 4 induction cycles followed by maintenance therapy until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care and premedication for chemotherapy. **Prohibited:** Other investigational drugs, concurrent systemic anti-cancer

therapies, and live-attenuated vaccines during the treatment period.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Scans, Labs, ECOG PS
Baseline	Day 1	Randomization, First Dose (Induction Phase Initiation)
Interim	Every 21 Days	IV Infusions, Safety Labs, Efficacy Assessment (Tumor imaging every 6-9 weeks)
End of Study	Upon PD/Death	Final Safety Follow-up, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Investigator-Assessed Progression-Free Survival (PFS) in the Primary Analysis Set (PAS): PAS includes patients without history/presence of brain metastases at baseline. PFS is the time from randomization to first documented PD as determined by the investigator using RECIST v1.1 or death from any cause. PD is defined as at least a 20% increase in the sum of diameters (SOD) of target lesions (using the smallest SOD on study as reference) plus an absolute increase of ≥ 5 mm.

Overall Survival (OS) in the PAS: Defined as the time from randomization to death from any cause.

7.2 Secondary & Exploratory Endpoints - PFS in the Full Analysis Set (FAS). - OS in the FAS. - Investigator-Assessed Confirmed Objective Response Rate (ORR) in the PAS (defined as CR or PR on two consecutive occasions ≥ 4 weeks apart). - Duration of Response (DOR). - PFS rate at 6 and 12 months; OS rate at 12 and 24 months.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection of all AEs and Treatment-Related Adverse Events (TRAEs) graded according to severity during site visits and continuous monitoring. **Serious Adverse Events (SAEs):** Evaluated continuously and reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent safety monitoring committee to evaluate safety and toxicity during the trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Primary Analysis Set (PAS; patients without brain metastases) and Full Analysis Set (FAS; all randomized patients). **Sample Size Justification:** $N=490$. The final analysis

of OS in the PAS was planned for when ~288 deaths were observed, providing 85% power to detect a target OS HR of 0.70 at a two-sided significance level of 0.049. **Handling of Missing Data:** Standard right-censoring methods for time-to-event endpoints (Kaplan-Meier estimates).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure protocol adherence. **Audit Trail:** eCRF locking, tracking of clinical pathway adherence, and independent radiologic review capabilities.

11. Study Identifiers & Governance

Primary ID: G041767 **Registry Number:** NCT04256421 **Posting ID:** 375870991005703224 **Sponsor/Lead Organization:** Hoffmann-La Roche / Roche **Study Title:** A Study of Atezolizumab Plus Carboplatin and Etoposide With or Without Tiragolumab in Patients With Untreated Extensive-Stage Small Cell Lung Cancer **Official Regulatory Title:** A Phase III, Randomized, Double-Blind, Placebo-Controlled Study of Atezolizumab Plus Carboplatin and Etoposide With or Without Tiragolumab (Anti-Tigit Antibody) in Patients With Untreated Extensive-Stage Small Cell Lung Cancer

12. Clinical Framework (Oncology Specific)

Primary Condition: Extensive-Stage Small Cell Lung Cancer (ES-SCLC) **Diagnosis Threshold:** Histologically or cytologically confirmed ES-SCLC (Measurable disease per RECIST v1.1) **Medical Methodology:** Standard of Care (Atezolizumab + CE) + Anti-TIGIT immunotherapy **Optimization Target:** Progression-Free Survival (PFS) and Overall Survival (OS) extension

13. Study Procedures & Methodology

Management Pathway: Standard clinical oncology pathway for ES-SCLC with 21-day (Q3W) cycles **Education Protocol (HCP):** Protocol training, RECIST v1.1 criteria application, and adverse event management **Education Protocol (Patient):** Onsite training regarding immunotherapy safety, infusion reactions, and toxicity self-reporting **Quality Audit Mechanism:** Investigator response assessments and centralized safety monitoring

14. Observation & Follow-Up Schedule

Total Duration: Up to 24 months (survival follow-up) **Onsite Visit Cadence:** Every 3 weeks (Day 1 of each 21-day cycle) for treatment infusions **Remote Monitoring Frequency:** Routine safety and survival follow-up **Checklist Review Interval:** Tumor assessments every 6-9 weeks

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					